



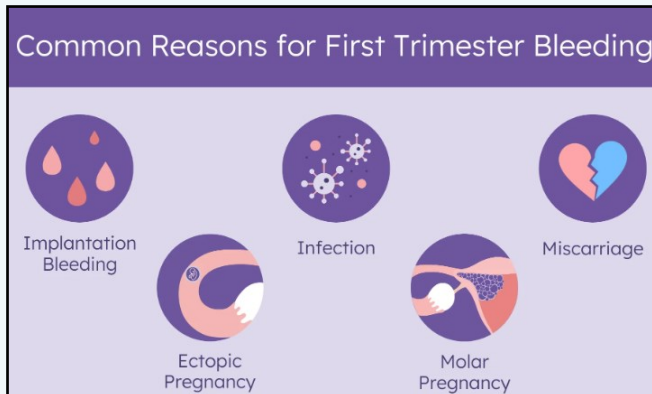
First Trimester Bleeding Management

Discovering the Causes of First Trimester Bleeding

Why Do Patients Experience First Trimester Bleeding?

Primary Causes

- Threatened AB: Normally progressing intrauterine pregnancy (IUP) with bleeding.
- Early Pregnancy Loss (EPL): Preferred term for fetal pole seen with no cardiac activity; outdated term “missed abortion (MAB).”
- Ectopic pregnancy
- Implantation bleeding: Spotting at time of missed period—up to 25% incidence.
- Cervicitis/vaginitis
- Other: Recent intercourse, cervical polyp, molar pregnancy, etc.



Note. From *Common Reasons for First Trimester Bleeding* [Infographic], by Amy Beckley, 2023, Proovtest (<https://proovtest.com/blogs/blog/pregnant-with-period>).

Important Facts to Consider



Note. From *Ectopic Pregnancy* [Photograph], by Efraim Hart, 2017, Wikipedia (https://en.wikipedia.org/wiki/Ectopic_pregnancy#/media/File:EUG_2.png)

- 15-25% of pregnancies will have 1st trimester bleeding.
- 10-40% of pregnancies have EPL (age-dependent).
- 2% of pregnancies are ectopic. (Ectopic pregnancies can kill young, healthy women.)
- 2.7% of pregnancy deaths are due to ruptured ectopic pregnancies.
- Up to 18% of 1st trimester pregnancies with bleeding may be ectopic pregnancies.
- Top risk factors for ectopic pregnancy include previous tubal surgery, previous ectopic pregnancy, h/o PID, current IUD use, and infertility.
- Heterotopic pregnancy risk is 1 in 100 (in vitro) to 1 in 30,000.

Navigating Visits with First Trimester Bleeding

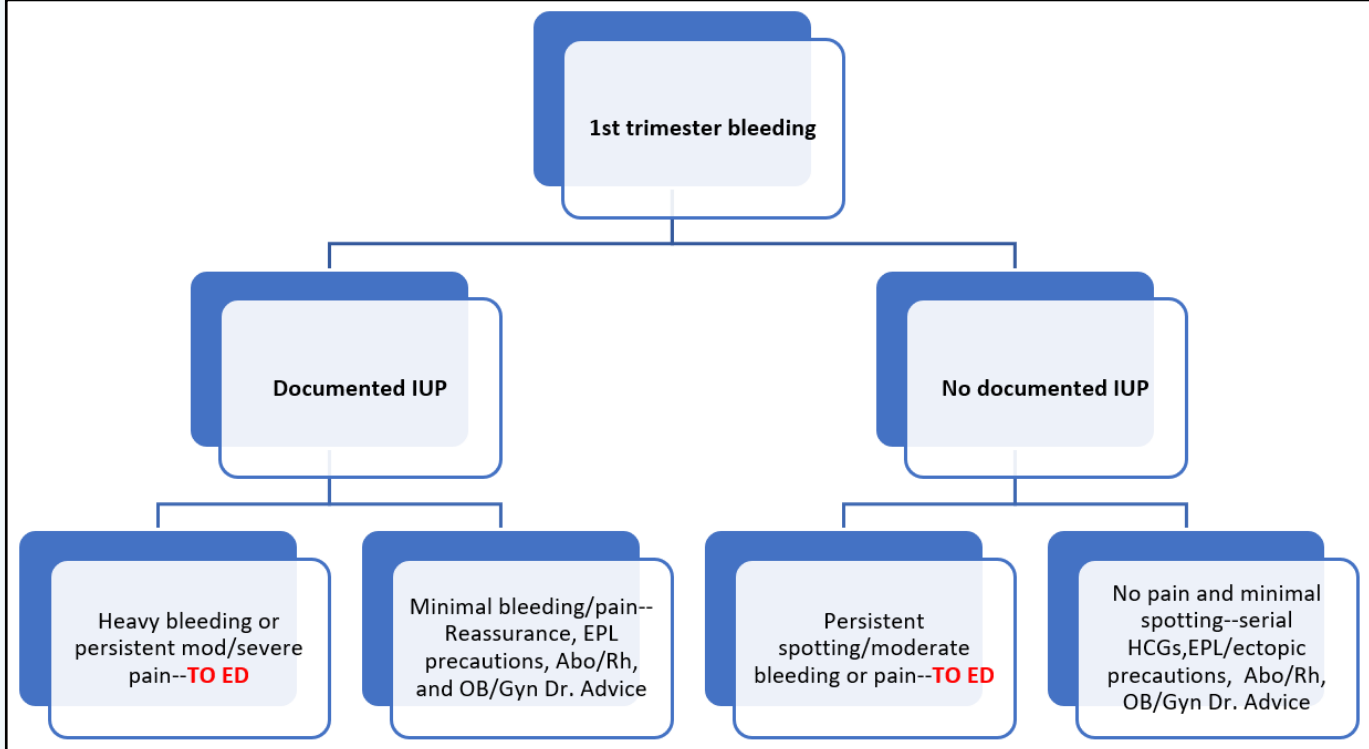
How Do I Navigate a Virtual Visit?

History is **Vital**.

- Has she had an ultrasound documented IUP (showing fetal pole and/or yolk sac)?
- LMP
- How much bleeding? Any signs of anemia (syncope, palpitations, severe fatigue)?
- How much pain?
- Any recent intercourse, vaginal discharge/itching, elevated risk for ectopic, etc.?

What Do I Do When a Patient Calls with First Trimester Bleeding?

Take the history above, then use the algorithm. Let's practice with case studies.



Case 1: 28 yo G1 at 8w3d by sure LMP. Calling VMC because of vaginal spotting x 2 days. She reports mild intermittent midline uterine cramping. Denies discharge, dysuria. Had intercourse a few days ago. Had her prenatal intake last week and has documented IUP with cardiac activity. Rh pos.

Dx: Threatened Ab. Documented IUP, minimal pain/bleeding: Ok for reassurance, EPL precautions, OB/Gyn Dr. Advice. No need for HCGs.

Case 2: 32 yo G2P1 at 6w1d by LMP. Calling VMC because of vaginal bleeding which started as spotting, now like a light period. Reports moderate cramping. No documented IUP. Rh neg.

Dx: ELP v. Ectopic v. threatened AB. No documented IUP. Has moderate pain and increasingly heavy bleeding. Advise to ED for further evaluation. Rh neg, but going to ED so no action needed for Rhogam via RVMC order.

Case 3: 42 yo G3P0 at 5w5d by IVF. Calling because of light spotting for 2 days. Pain mild in LLQ. Prior ectopic pregnancy with left salpingectomy. No documented IUP yet. Rh+.

Dx: Elevated suspicion for ectopic (IVF, h/o ectopic). LLQ pain. Advise to go to ED for further evaluation.

Case 4: 23 yo G1 at 6w2d by LMP. Calling because of spotting for 2 days, pink when she wipes. Has vulvar itching and an increase of whitish/yellow discharge. Pain-free. Has new OB intake next week, no US yet. No ectopic risk factors. Unknown Abo/Rh.

Dx: Spotting due to vaginitis v. threatened AB v. EPL v. Ectopic. *Order HCG today and in 48 hours.* Also order vaginitis panel and gc/ct due to vaginitis symptoms. Follow these results and assure a rise by at least 50%. Ectopic/EPL precautions. OB/Gyn Dr. Advice. Order Abo/Rh.

Understanding HCG Trends

What Should We Look Out For?

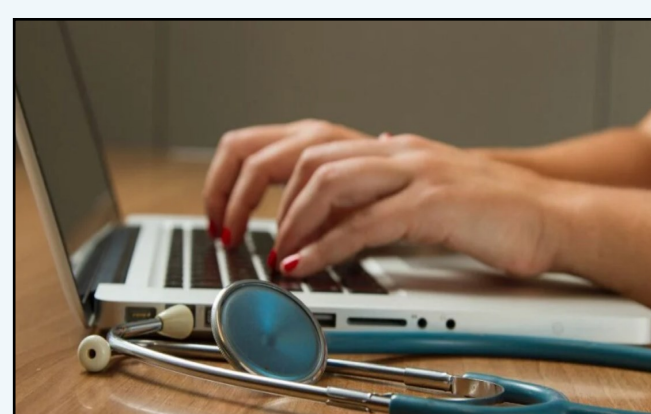
- Serial hCGs are used to differentiate normal from abnormal pregnancies.
- An increase in serum hCG of less than a minimal threshold in 48 hours is suspicious of an abnormal pregnancy (ectopic or early pregnancy loss).
- The expected rate of increase is:
 - 49% for an initial hCG level of less than 1,500 mIU/mL
 - 40% for an initial hCG level of 1,500 – 3,000 mIU/mL
 - 33% for an initial hCG level greater than 3,000 mIU/mL
- Approximately 95% of women with a spontaneous early pregnancy loss will have a decrease in hCG concentration of 21 – 35% in 2 days depending on initial hCG levels.

Additional Reading

Other Great RVMC First Trimester Bleeding Resources

- Dr. Anu Singh's [OB Triage Clinical Corner](#).
- Dr. Adetuni Adegboyega and Dr. Caroline Snarskis' [OB triage presentation](#).
- OB SmartPhrases developed by Dr. Adetuni Adegboyega and Dr. Caroline Snarskis
 - .VMCOBTriageBLEEDING1STTRIMESTERHX
 - .VMCOBTriageBLEEDING1STTRIMESTERPLAN
- Smart Set:** “Pregnancy of Unknown Location”
- Reach out for [OB/Gyn phone consult](#).
- Reach out to me. I am happy to chat.

Introducing the GPC Mailbag!



At GPC Clinical Corner, we strive to provide tips on the best clinical practices, navigating patient encounters, and updated information on treatment options.

This year, we'd like to expand our scope to include your suggestions. We're proud to announce the GPC Mailbag, where you can request topics for discussion, or even specific questions, that you'd like to see in a future newsletter. You can reach us at GPCMailbag@kp.org. We look forward to hearing from you!

References

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- ACOG. (2018, November). ACOG practice bulletin No. 200: Early pregnancy loss. ACOG. https://journals.lww.com/greenjournal/fulltext/2018/11000/acog_practice_bulletin_no_200_early_pregnancy.41.aspx
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